PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Subjective experiences of the response to mental health crises in the community: A qualitative systematic review.
AUTHORS	Xanthopoulou, Penny; Thomas, Ciara; Dooley, Jemima

VERSION 1 – REVIEW

REVIEWER	Webb, Lucy Manchester Metropolitan University
REVIEW RETURNED	30-Sep-2021

GENERAL COMMENTS	Thank you for submitting this systematic review to BMJ Open. This is a very important research area given the pressures on mental health services and issues in community care, and pressures on emergency services currently. The key findings in this review are relevant to policy and practice to address these pressures and should be disseminated. The procedure used here is relevant to a qualitative systematic review and follows the PRISMA reporting procedures well. However, there are several points that could be addressed to improve this paper. It is laudible that PPI is included but it is not clear how this input has been used. this needs explaining, especially if this input has informed the justification and search strategy or the analysis. It is not clear why a wide international range of studies has been used when he justification is largely centred on UK practice. It is good to aim for generalisability of the findings, but it is likely that the evidence from heterogeneous sources and cultural contexts would result in reduced applicability to the UK context. The themes are well presented and supported by evidence, however the interpretation of the evidence occasionally goes beyond the evidence. for example, that improving communication with carers is not supported by this evidence as claimed in the abstract. Interpretations need to be stated as suppositions rather than fact in your concluding sections. A stated aim appears to be to fill the gap in evidence around successful communication strategies in MHC encounters between first responders and patients and carers in the community setting, and this is fine, but the methodology does not indicate any a priori themes however interpretation of the findings does appear to explore this element. This is a little 'tail wagging the dog' as there are some assumptions that the evidence does not sufficiently support. A clear finding is however, as you state, that confusion over the services being appropriate for this role is supported by the evidence and more convincing discussion could be made of this finding. There are several typos and grammatical/punctuation errors. I have indicated some of these on the attached script.

REVIEWER	Phan, Phillip Johns Hopkins University
REVIEW RETURNED	08-Oct-2021

GENERAL COMMENTS	Thank you for giving me the opportunity to review the paper. I hope the following comments are useful for your work. 1. there is much going on in the introduction. You cite policy and implementation issues related to FR interactions with PMI. I think you need a clearer statement of the research question and its intended contribution to the literature. This is not a policy paper, per se, and your review does not focus on policy papers. Therefore, you should confine your research question and recommendations to the domains represented in the literature you surveyed, which are related to implementation issues. 2. You used PICo and not PICO. You did not explain why you did not include Outcomes in your review, since the purpose was to improve FT/PMI interactions, which, by definition, includes outcomes as the measure of effective or non-effective interactions 3. The most serious concern I have is that you excluded quantitative studies. Studies that use large sample surveys measuring attitudes and affects are also relevant, it seems to me, and could represent more rigorous ways of assessing lived experiences. Please explain why you excluded all quantitative studies. 4. You mentioned that 66 studies were considered high quality whilst the rest were intermediate quality. Please define what you mean by quality (please provide a more detailed discussion how you used CASP to assess study quality), and provide a procedural map of how you formally analyzed quality or indicate in Table 1, which studies were high quality and why in the a 'notes' column. I think you can appendicize Table 1 if it makes the paper too long. 5. Two reviewers were used to conduct the screening. Did these same 2 reviewers also do the thematic extraction using nVivo? If so, how did you guard against bias? Generally, it's best that the same individuals extracting the papers for review not be the same individuals extracting the themes since there would be a confirmation bias built into such a process. 6. You provided a long list of countries in which the studies were conducted. What is the purpose of the list? Did you find systematic variation in the way FR/PMI interactions occured? If so, you can say something about these variations (were they due to culture, regulation, norms and values)? 7. You presented a list of themes in the results and discussed them as independent of each other. They are likely related to each other in important ways. Can you provide a picture of those relationships and discuss the relationships among the themes so the reader can benefit from the insights of your deep dive into the data? The qualitative papers may have suggested relationships among the themes they found, how do these vary among the papers you reviewed? The presentation of the results do not mirror the complexity of the phenomenon implied in your introduction and conclusion.

REVIEWER	Barr, Karlen University of Wollongong, Psychology
REVIEW RETURNED	14-Oct-2021

GENERAL COMMENTS	Comments to the Author Overall, I think this manuscript importantly illustrates how people with lived experience of mental health difficulties face mistreatment from first responders which can increase their distress and trauma. I appreciate the inclusion of the carer perspective and how limited communication negatively influences carers. The call for more training of first responders to more compassionately support people with mental health problems is needed. In the abstract, rather than listing the themes that were developed, I recommend listing some more precise findings from the results section to help the reader understand what the main findings of the research were. The abbreviation 'MH' needs to be defined in the abstract and in the introduction. In the second sentence, I suggest you write full name of NHS as international readers may not know what you are referring to. The last sentence of the first paragraph needs grammatical changes i.e., '…where the police respond, they should…' Sentence two of paragraph two -check reference (a 5 is listed here but not written in superscript). On page 4 line 31, 'Ambulance Service: 4' – change 4 to written four to assist the reader. The Screening, data extraction and synthesis section of the method section requires more information about how thematic synthesis was performed. For example, what steps were involved in the synthesis and was the synthesis done using an inductive or deductive approach. It would be helpful to define the term 'companion' within the text. I can see that it is listed in the abbreviation section, but while reading the text it is unclear whether this term refers to carers/family. I am not sure the subtheme 'Force and physical restraint: criminalisation vs recognition' is appropriately titled. It reads as though some people may feel recognised through force and physical restraint, whereas what I believe you are trying to communicate is that PMI feel valued when they are recognised and receive communication from first responders. Page 12 line 6 "crises was not considered a 'real' emergency" needs to be changed grammatically to 'crises were not…' And line 11 'These encounters often resulted the PMI feeling ashamed' needs to be changed to 'resulted in the…' Page 13 line 46 "PMI believed that even a gentle response from police, without MHPs present, was irrelevant because as police

	are not qualified" needs some grammar changes i.e., 'irrelevant because police…' On page 14 line 39 "within first police and paramedic services" – it is unclear why 'first' is written. I suggest removing 'first' or adding 'first responder'. At the beginning of the discussion, I recommend reorienting the reader to what this study was about and what the aim was, before discussing the findings. I think the recommendation to include PMI and their carers in mental health training for first responders is a great idea. In the limitations section, I think it would be important to state that this study did not discuss specific mental health problems or disorders. It would be interesting to know whether first responders treated people with different diagnoses or presenting problems differently, and how this impacted PMI and their carers. For example, people with personality disorder often experience stigma and mistreatment from first responders. Given that it is possible that people with lived experience may read this paper, it may be beneficial to give a language warning (e.g., in the acknowledgement section) as there are some comments written by first responders that may be distressing for people with lived experience to read.

REVIEWER	Chuang, Hung-Yi Kaohsiung Medical University Hospital, Department of Occupational and Environmental Medicine
REVIEW RETURNED	20-Oct-2021

GENERAL COMMENTS	This review article used PRISMA 2020 guideline to search the research reports related "stakeholder experiences of the response to community-based mental health crisis". Finally, they found 79 full-text papers for this qualitative systematic review. They concluded "The lack of resources in mental healthcare and rise in mental illness, suggests that FR response to MHC is inevitable. Inconsistent training, complexity of procedures and persistent stigmatisation make this a very challenging task. Improving communication with family carers and colleagues can make a difference. Broader issues of legitimacy and procedural barriers should be considered in order to reduce criminalisation and ensure an empathetic response." Since they used a widely accepted method for this systemic review, the structure and materials were fine to me. However, they discussed "stakeholders" and concluded "lack of resources …". I would suggest to analyze how different among countries/areas, how much difference between private and public sections of stakehoders.

VERSION 1 – AUTHOR RESPONSE

Response to Reviewers Comments

We thank the reviewers for their very helpful comments. Please find the comments and our responses below, detailing the changes in the revised manuscript.

Reviewer: 1

1. It is laudable that PPI is included but it is not clear how this input has been used. This needs explaining, especially if this input has informed the justification and search strategy or the analysis.

Thank you for this comment. The following was added to the PPI section (p.3): FR response was described by carer and PMI as “hit and miss”, “dismissive” but also positive “they [police] always looked after me”. FRs described the challenges they face and lack of, or difficulties in following prescriptive training. This led to development of aims of this review and the focus on real-life experiences.

2. It is not clear why a wide international range of studies has been used when the justification is largely centred on UK practice. It is good to aim for generalisability of the findings, but it is likely that the evidence from heterogeneous sources and cultural contexts would result in reduced applicability to the UK context.

Thank you for highlighting this. Potential differences across different countries both in policing and mental health services, are indeed important. We have now added the countries where studies were included for each sub-theme in Table 2. Themes and subthemes, where we can see that analytic patterns of experiences run across different countries. However, we cannot make further assumptions regarding this.
We have also added the following text to highlight this (Strengths and limitations section): Differences between countries was not systematically explored. Despite potential significant differences in policing and mental health services in the countries where included studies were conducted, analytic patterns of experiences (themes and subthemes) were found across different countries. However, the majority of these studies were in developed countries and studies not reported in English were not included. Further research could explore if there is variation in the response, by country with regards culture, regulations, norms etc.

3. The interpretation of the evidence occasionally goes beyond the evidence. for example, that improving communication with carers is not supported by this evidence as claimed in the abstract. Interpretations need to be stated as suppositions rather than fact in your concluding sections.

Theme 4, Impact of response on companions: involvement hindering vs facilitating response, describes finding in relation to information sharing between carers and FRs, specifically text: Failure to communicate with PMI/companions worsen the crises: “If they had sat down with me instead and said ‘okay, do you want to tell us what’s been going on…it would have been easier and they wouldn’t have needed the handcuffs because I wouldn’t have ran” (companion).[26]
Therefore, we believe that this evidence supports this finding however we have revised it as: Improving communication with family carers and colleagues can could make a difference.

4. A stated aim appears to be to fill the gap in evidence around successful communication strategies in MHC encounters between first responders and patients and carers in the community setting, and this is fine, but the methodology does not indicate any a priori themes however interpretation of the findings does appear to explore this element. This is a little 'tail wagging the dog' as there are some assumptions that the evidence does not sufficiently support.

Thank you for highlighting this inconsistency. This text in the background has been changed from: Currently, evidence on successful communication strategies for emergency services is not well documented nor integrated into routine practice.
to: Currently, evidence on FR interactions and communication practices in incidents involving people with mental ill health is not well documented.

5. A clear finding is however, as you state, that confusion over the services being appropriate for this role is supported by the evidence and more convincing discussion could be made of this finding.

Thank you for this suggestion. We are now discussing this further in the discussion section: However, the role of first responders in managing mental ill health in the community is not only vital but will potentially increase due the Covid-19 pandemic[103] and the subsequent economic crisis, as well as climate change, which disproportionally affects vulnerable people and can be the cause for mental illness.[104] Similarly, although interagency collaboration models where mental health professionals are located in police stations or police cars, can improve response,[18] most of these models are not evaluated, and high costs, logistics and limited access to local mental health services make such interventions difficult to implement.[105]

6. There are several typos and grammatical/punctuation errors. I have indicated some of these on the attached script.

Thank you for highlighting this. We have now corrected these.
Remark on the marked reviewer manuscript copy: “mixed experiences but labelled as negative.” Regarding P12, lines 25-33., the one experience described as good is not interpretated as ‘positive’ experience and explained: “This was also demonstrated in the low standards PMI had for interactions with FRs: “I didn’t get beat up, I didn’t get thrown down stairs, I didn’t get punched or kicked, I didn’t get tazed, that kind of stuff, so, it was good” (PMI).35

Reviewer: 2

1. There is much going on in the introduction. You cite policy and implementation issues related to FR interactions with PMI. I think you need a clearer statement of the research question and its intended contribution to the literature. This is not a policy paper, per se, and your review does not focus on policy papers. Therefore, you should confine your research question and recommendations to the domains represented in the literature you surveyed, which are related to implementation issues.

Thank you for highlighting this, we have removed some of the local/UK guidance with international - World Health Organisation (1st paragraph of introduction, and refence no.6).

2. You used PICo and not PICO. You did not explain why you did not include Outcomes in your review, since the purpose was to improve FT/PMI interactions, which, by definition, includes outcomes as the measure of effective or non-effective interactions.

The purpose of this review was to explore people's experience of the response, rather than directly improve the response. We use PICo, which is commonly used for these kinds of qualitative reviews, where we don't measure specific outcomes. We agree that this may not have been straightforward and have changed the title and text to reflect this purpose better.

3. The most serious concern I have is that you excluded quantitative studies. Studies that use large sample surveys measuring attitudes and affects are also relevant, it seems to me, and could represent more rigorous ways of assessing lived experiences. Please explain why you excluded all quantitative studies.

We have used a qualitative only studies our aim was to provide a synthesis of qualitative literature regarding the challenges experienced by all stakeholder and hence focus on subjective experiences. A qualitative review can help us focus on the meanings and interpretations of the participants.

- To clarify this, we have changed the title from: "Stakeholder experiences of the response to community-based mental health crisis: A qualitative systematic review."

To: "Subjective experiences of the response to mental health crises in the community: A qualitative systematic review."

- We have also changed the aim of the study in the abstract and the background: from "…perspectives of stakeholders" to "...subjective experiences of people..."

4. You mentioned that 66 studies were considered high quality whilst the rest were intermediate quality. Please define what you mean by quality (please provide a more detailed discussion how you used CASP to assess study quality) and provide a procedural map of how you formally analyzed quality or indicate in Table 1, which studies were high quality and why in the a 'notes' column. I think you can appendicize Table 1 if it makes the paper too long.

Thank you for your suggestion to add this. We have

1. Added an extra column to table 1 with the CASP score for each study
2. Added further explanation to the results section (1st paragraph) that related to CASP: All scored 7 or higher on the CASP Checklist (see table 1), indicating medium to high quality; no exclusions were made on this basis. Reasons for lower scores were: a) not reporting on the relationship between researchers and participants, b) limited reporting on ethical issues and c) not detailed reporting on the analysis.

5. Two reviewers were used to conduct the screening. Did these same 2 reviewers also do the thematic extraction using nVivo? If so, how did you guard against bias? Generally, it's best that the same individuals extracting the papers for review not be the same individuals extracting the themes since there would be a confirmation bias built into such a process.

The same 2 reviewers conducted the analysis in analytic meetings. Although it would have been ideal to include another 2 people in this study, this was not possible due to lack of resources. It is however common that 2 reviewers are involved in the screening, extraction and analysis in systematic reviews.

6. You provided a long list of countries in which the studies were conducted. What is the purpose of the list? Did you find systematic variation in the way FR/PMI interactions occurred? If so, you can say something about these variations (were they due to culture, regulation, norms and values)?

Thank you for this comment. Data on mental health resources for each country included in this review is not reported in the original analysis and therefore we cannot report that. We cannot derive to any conclusion based in the findings we have as differences between countries was not systematically

explored. However, we have now added the countries where studies were included for each sub-theme in Table 2. Themes and subthemes, and in limitations section: Differences between countries was not systematically explored. Despite potential significant differences in policing and mental health services in the countries where included studies were conducted, analytic patterns of experiences (themes and subthemes) were found across different countries. However, the majority of these studies were in developed countries and studies not reported in English were not included. Further research could explore if there is variation in the response, by country with regards culture, regulations, norms etc.

7. You presented a list of themes in the results and discussed them as independent of each other. They are likely related to each other in important ways. Can you provide a picture of those relationships and discuss the relationships among the themes so the reader can benefit from the insights of your deep dive into the data? The qualitative papers may have suggested relationships among the themes they found, how do these vary among the papers you reviewed?

Thank you for this comment. Although it may be the case that for a few of the study some of the subthemes might be related, this was not a pattern identified across the data of a significant number of studies, many of which focused on one specific aspect, e.g., use of force. However, in the discussion we make such links, e.g., use of force and stigma: “interactions involved excessive force, physical restraint and stereotyping, which often humiliated and traumatised PMI and their companion”.

We have however, changed the first paragraph of the discussion to reflect the potential links between the subthemes: Recognising mental illness as a legitimate condition and responding with empathy rather than force and stigmatising attitudes was valued by PMI and their companions. To achieve this, all stakeholders called for further training. FRs highlighted issues regarding the law, procedures, and accountability which may constrain the response. Despite the inevitability of their involvement in these incidents, many FRs do accept they should have a role in managing mental illness in the community. Personal experience of mental illness can make a difference in the response as well as collaboration between first responder services and mental health services. Finally, the role of 3rd parties present in the interaction, such as family, friends and carers, was recognised by all stakeholders, which adds another level of complexity to these interactions.

Reviewer: 3

1. In the abstract, rather than listing the themes that were developed, I recommend listing some more precise findings from the results section to help the reader understand what the main findings of the research were.

Thank you for this suggestion. We agree and have revised the abstract to reflect this.

2. The abbreviation ‘MH’ needs to be defined in the abstract and in the introduction.

Thank you for this comment. We have now corrected this.

3. In the second sentence, I suggest you write full name of NHS as international readers may not know what you are referring to.

We have added the full name.

4. The last sentence of the first paragraph needs grammatical changes i.e., ‘…where the police respond, they should…’

We have now rephrased to the following to aid clarity.

5. Sentence two of paragraph two -check reference (a 5 is listed here but not written in superscript).

We have now corrected this, thank you.

6. On page 4 line 31, 'Ambulance Service: 4' – change 4 to written four to assist the reader.

This section is now corrected and revised.

7. The Screening, data extraction and synthesis section of the method section requires more information about how thematic synthesis was performed. For example, what steps were involved in the synthesis and was the synthesis done using an inductive or deductive approach.

Thank you for this suggestion, we have now added further information in the methods section.

8. It would be helpful to define the term 'companion' within the text. I can see that it is listed in the abbreviation section, but while reading the text it is unclear whether this term refers to carers/family.

We have added text on the last paragraph of the introduction the following: "…including companions (family/carers/friends)."

9. I am not sure the subtheme 'Force and physical restraint: criminalisation vs recognition' is appropriately titled. It reads as though some people may feel recognised through force and physical restraint, whereas what I believe you are trying to communicate is that PMI feel valued when they are recognised and receive communication from first responders.

Thank you for this comment we agree and have rephrased this to: Criminalisation vs recognition of behaviour relating to mental ill health, in the table and in the results.

10. Page 12 line 6 "crises was not considered a 'real' emergency" needs to be changed grammatically to 'crises were not…' And line 11 'These encounters often resulted the PMI feeling ashamed' needs to be changed to 'resulted in the…'

We have now corrected these, thank you.

11. Page 13 line 46 "PMI believed that even a gentle response from police, without MHPs present, was irrelevant because as police are not qualified" needs some grammar changes i.e., 'irrelevant because police…'

We have now corrected this, thank you.

12. On page 14 line 39 "within first police and paramedic services" – it is unclear why 'first' is written. I suggest removing 'first' or adding 'first responder'.

We have now corrected this - removed first, thank you.

13. At the beginning of the discussion, I recommend reorienting the reader to what this study was about and what the aim was, before discussing the findings.

Thank you for this suggestion. We have added the following at the beginning of the discussion: This review of the qualitative aimed to identify and synthesise findings from published studies on the subjective experiences of stakeholders involved in incidents/crises which involve people with mental ill health.

14. In the limitations section, I think it would be important to state that this study did not discuss specific mental health problems or disorders. It would be interesting to know whether first

responders treated people with different diagnoses or presenting problems differently, and how this impacted PMI and their carers. For example, people with personality disorder often experience stigma and mistreatment from first responders.

Thank you for this comment. The following was added to the limitations section: This study did not explore specific mental health problems. Further research could investigate whether first responders' response to people with different diagnoses varies and how.

15. Given that it is possible that people with lived experience may read this paper, it may be beneficial to give a language warning (e.g., in the acknowledgement section) as there are some comments written by first responders that may be distressing for people with lived experience to read.

Thank you for this suggestion. We have now added a sentence reflecting this in the Ethics statement section: Some people might find direct quotes presented in this article distressing.

Reviewer: 4

1. Since they used a widely accepted method for this systemic review, the structure and materials were fine to me. However, they discussed "stakeholders" and concluded "lack of resources …". I would suggest to analyse how different among countries/areas, how much difference between private and public sections of stakeholders.

Thank you for this comment. Data on mental health resources for country reported in this review is not reported in the data and therefore we cannot report that. However, we have now added the countries where studies were included for each sub-theme in Table 2. Themes and subthemes and added text (see reviewer 1 comment 2).

VERSION 2 – REVIEW

REVIEWER	Phan, Phillip Johns Hopkins University
REVIEW RETURNED	16-Dec-2021

GENERAL COMMENTS	Thank you for resubmitting your manuscript for further review. Although I did not receive your response to the editor and reviewers, my reading of the manuscript suggests no further concerns.

REVIEWER	Barr, Karlen University of Wollongong, Psychology
REVIEW RETURNED	20-Dec-2021

GENERAL COMMENTS	Well done on making the changes suggested.

REVIEWER	Chuang, Hung-Yi Kaohsiung Medical University Hospital, Department of Occupational and Environmental Medicine
REVIEW RETURNED	17-Dec-2021

GENERAL COMMENTS	I have no more comments.